



Delirium

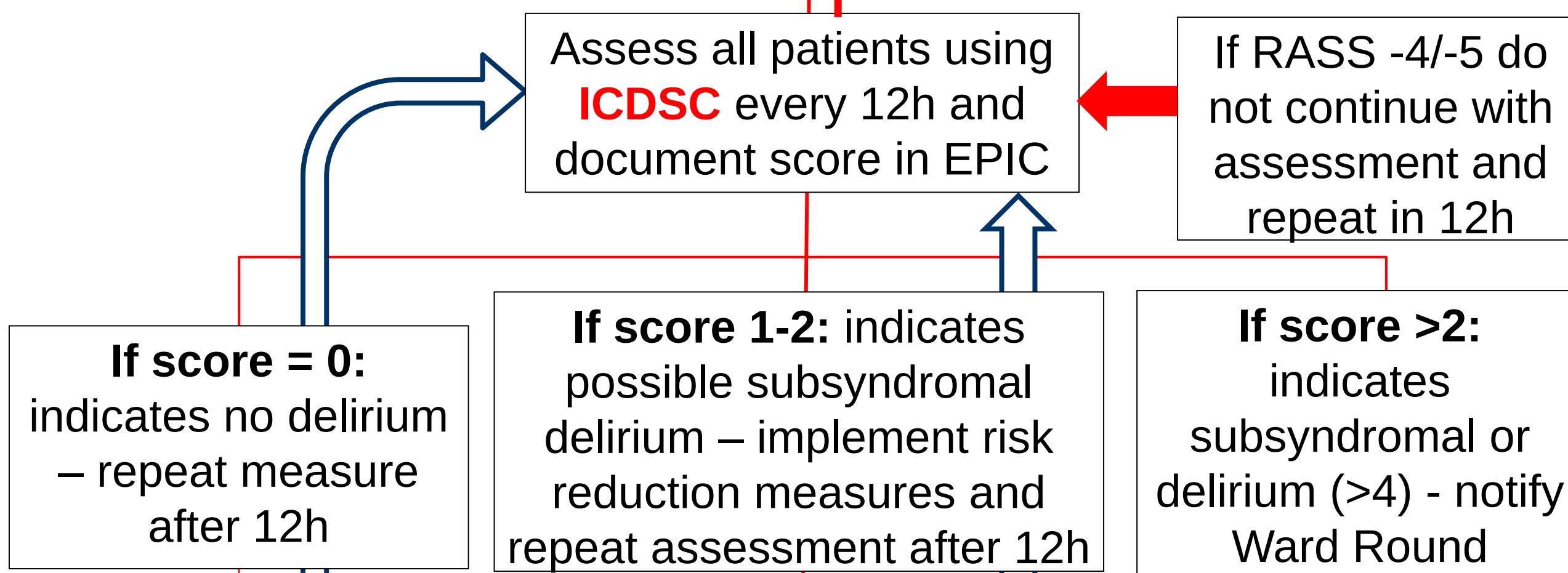
Delirium is defined as a disturbance of consciousness and cognition that develops over a short period of time (hours to days) and fluctuates over time
It is a common manifestation of acute brain dysfunction in critically ill patients

ASSESSMENT and MANAGEMENT

In some patients **agitation** may be a manifestation of delirium

RISK FACTORS

Dehydration
Hypoxaemia
Pain
Infection
Drugs
Prolonged ICU stay
Prolonged ventilation
Visual / hearing impairment
Cognitive impairment
Immobility
Advancing age



Further assessment: Symptoms which are not better explained by a pre-existing, established or evolving neurocognitive disorder and do not occur in the context of a severely reduced level of arousal such as coma (DSM-V Crit D)

If confirmed (>4) add to diagnosis in EPIC

Complete smart text checklist

PINCHES ME

Implement risk reduction and treatment strategies

If other cause for current symptoms or change in presentation suspected / identified then investigate appropriately e.g. CTH / EEG

If hyperactive delirium where patient is agitated and poses risk to self or others consider:

- Rationalising unnecessary lines / interventions
- Use of additional nursing
- Physical / mechanical restraints*
- Drug therapies (see separate guidance)

*Least restrictive measure to be utilised in line with level of risk

Provide information and psychoeducation to family

ENVIRONMENTAL FACTORS

Environmental changes where possible

Noise reduction
Day - ↑ exposure to natural light
Night - ↓ exposure to artificial light
Optimum ambient temperature
Improved communication
Repeat orientation to day, time, place

PHARMACOLOGY

for **AGITATION**

for **SLEEP**

Haloperidol
Risperidone
Clonidine
Olanzapine
Dexmedetomidine

Melatonin
Zopiclone
Chloral hydrate

Avoid benzodiazepines

Think Delirium
RISK REDUCTION
ASSESS and TREAT
The common causes of Delirium
Remember **PINCHES ME**:

- Pain
- Infection
- Nutrition
- Constipation
- Hydration
- Environment
- Sleep
- Medication
- Exercise (may help protect against Delirium)

For more information, please email Vicky MacRae, Delirium nurse: victoria.macrae@aph.nhs.uk #WDAD2019

LOOK FOR REVERSIBLE CAUSES

Intensive Care Delirium Screening Checklist
For any component of the checklist, if you are unable to assess, answer No = Score 0
A total ICDSC score greater or equal to 4 has a 99% sensitivity for a psychiatric diagnosis of delirium.

Altered level of consciousness	Assessment Considerations
RASS = +1 to +4 Exaggerated response	Score = 1 Assess level of consciousness at the time of ICDSC scoring
RASS = 0 Normal wakefulness / calm / cooperative	Score = 0 May need to delay assessment if patient is sedated or has recently administered sedatives, score for patient's current condition.
RASS = -1 to -2 Responds to mild stimulation	Score = 1
RASS = -3 Responds to moderate stimulation	Score = 1
RASS = -4 to -5 Responds only to intense repeated stimulation OR No response to noxious stimulation	STOP ASSESSMENT

Inattention

Assessment	Assessment Considerations
Difficulty following simple commands	Yes = Score 1 Attention needs to be held for a minimum of 10 seconds.
Attentive and focused	No = Score 0 Does the patient have the ability to organize their thoughts?
Unable to assess	No = Score 0 Does the patient have difficulty focusing attention or difficulty tracking you?

L Click in NORMAL VIEW mode to view

Disorientation

Assessment	Assessment Considerations
Disoriented to person, place or time	Yes = Score 1 For inattentive patients use easy yes/no questions. Can the patient recognize family/caregivers? Do they know what kind of place they are in (hospital)?
Oriented or unable to assess	No = Score 0

Hallucination, delusion or psychosis

Assessment	Assessment Considerations
Visual, auditory or tactile hallucinations	Yes = Score 1 Hallucinations: Perception of something in the absence of stimuli.
Delusions	Yes = Score 1 Delusions: False beliefs with no feasible/reasonable reason.
Psychosis	No = Score 0 Psychosis: Difficulty telling what is real and what is not. Do you hear someone speaking to you other than me? Do you believe someone is trying to harm you?
No apparent hallucinations, delusion or psychosis or unable to assess	No = Score 0

Psychomotor agitation or retardation

Assessment	Assessment Considerations
Agitation or retardation	Yes = Score 1 Hypervigilance: Heightened arousal. Can be restless, agitated or aggressive.
Relaxed and cooperative or unable to assess	No = Score 0 Hypoactivity: Flat affect, withdrawn, decreased responsiveness, slowed speech, and/or apathetic.

Inappropriate mood or speech

Assessment	Assessment Considerations
Inappropriate mood, disorganized thoughts or inappropriate shouting	Yes = Score 1 Is the patient's speech or mood appropriate to the current situation?
Appropriate speech/mood or unable to assess	No = Score 0 Consider asking family/friends if this is typical for the patient.

Sleep wake cycle disturbance

Assessment	Assessment Considerations
Slept more than 4 hours total during the day	Yes = Score 1 Based on primary caregiver assessment within the past 24hrs.
Slept less than 4 hours total during the night or frequent waking	Yes = Score 1
Sleeping at least 4 hours at night or unable to assess	No = Score 0

Fluctuations

Assessment	Assessment Considerations
Worsening of any indicators in the last 24 hours (see previous shift)	Yes = Score 1 Worsening of an indicator which is not related to an intervention.
No change or improvement of delirium indicators	No = Score 0 For example, patient is less restless due to sedative for procedure.

Implement risk reduction measures

- Regular orientation
- Check vision / hearing aids are with patient
- Optimise sleep (see *sleep hygiene*)
- Early mobilisation if safe
- Avoid ward / bed moves if possible
- Family involvement
- Optimise communication
- Use of 'about me' activities
- Ensure appropriate level of stimulation
- Support a routine

Psychological care

- Promote sense of safety
- Use of "about me" to promote person centred care
- Assess and treat mood disturbance and agitation
- Psychoeducation for patient and family
- Relaxation
- Therapeutic activity

Communication

- Body language open, visible
- Avoid sudden movements
- Avoid gestures of ordering or criticising
- Caring tone of voice, manner
- Suggesting, not ordering
- Break up long messages
- Give time to respond
- Repeat if necessary
- Confirm you have understood
- Hold frustrations
- Do not contradict / argue especially with delusional content
- Slow down your speech
- Keep it clear and simple – use short and simple sentences



Delirium STANDARDS

ASSESSMENT

- Each patient will have a twice daily assessment completed by a trained nurse using the Intensive Care Delirium Screening Checklist (ICDSC)
- ICDSC score will be recorded in the “NHNN ICU Obs” section of *flowsheets* on EPIC
- Where a subsyndromal or a positive delirium score (>2) is identified this must be escalated to the ward round for more detailed assessment
- If confirmed, the diagnosis should be recorded in EPIC

TRAINING

All staff will be trained in the assessment, prevention and management of delirium

- Training will include:
 - What is delirium?
 - Delirium in the context of brain injury
 - Assessment using ICDSC
 - Prevention of delirium
 - Escalation protocol
 - Interventions for delirium
- Initial training will be conducted by members of Delirium Working Group (DWG) to delirium link nurses
- Further nurse training will be led by link nurses with support of DWG

GUIDANCE and GENERAL MANAGEMENT

- Guidance will be available regarding escalation through a smart text added to the patient’s notes once diagnosis has been confirmed
- Smart phrase in EPIC will include note for positive assessment and checklist to include:
 - Consideration of acute life threatening causes of reduced arousal
 - Screening for causes of delirium (PINCHES ME)
 - Non-pharmacological interventions
 - Communication strategies
 - Accessing specialist advice
 - Managing agitation
 - Ongoing monitoring

MEDICATIONS

To include:

- Dexmedetomidine (see protocol)
- Review / rationalisation of sedation on discharge
- Review of drugs that increase risk of delirium
- Managing withdrawal (illicit and analgesia)
- Sedation and sedation holds
- Treatment of agitation

SPECIALIST ADVICE

Psychiatry involvement for complex cases

DELIRIUM CARE BUNDLE

All patients to be on the delirium care bundle:

- All patients should have a twice daily screen for delirium using ICDSC that is recorded in EPIC
- All patients should have a sedation plan that includes sedation breaks *unless contraindicated*
- All patients should have a plan for mobilisation *unless contraindicated*
- All patients should have an “about me” completed and presented at bedside
- All patients should have a daily pain assessment that is recorded in EPIC
- All patients should have communication aids available when required
- All patients should be able to see a clock from their bed space
- All patients should have access to a sleep pack
- Medical interventions and observations should be reviewed and rationalised as permitted
- Where pharmacological or mechanical restraint interventions are required, these interventions should be reviewed daily

COMMUNICATION with RELATIVES

If there is a suspicion of delirium or a positive diagnosis is made then the family should be informed and provided with:

- Psychoeducation about delirium
- Discussion about how best to psychologically respond to the person and to discuss any specific needs
- Information leaflets on delirium and how to access further support (<https://www.rcpsych.ac.uk/mental-health/problems-disorders/delirium>)
- Discussion of potential treatment options
- Information on how to support a person with delirium

HANDOVER and RECORDING

Recording on EPIC and adding diagnosis on EPIC
Handover of diagnosis on transfer to the ward

AUDIT and EVALUATION

Regular audit of incidence and treatments

First published: Sign off text style
Date last reviewed:
Date next review due:
Leaflet code:
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How you can support
Contact with family members can be really helpful in supporting someone with delirium.
You can help someone with delirium by:

- Talking to them in short, simple sentences
- Talk to them about things they know and enjoy
- Offer reassurance and remind them of what is happening and where they are.
- Remind them of the time and date
- Bring in some familiar objects from home and photos of loved ones
- Make sure they have their glasses and hearing aids with them
- Help them to eat and drink
- Keep a diary for them of events
- Avoiding long tiring visits, loud conversations, and multiple visitors at any one time

To get know our patients better we have an #aboutme leaflet that families can fill out. This is very helpful in supporting us to get to know your loved one better. It will also help us understand their likes and dislikes and what things may be helpful in calming them if they are feeling upset or distressed. You can ask a member of the team for a form.
It can be difficult for relatives to witness their loved one behaving differently, so please do speak to the team if you have any questions. There is further support for families through the neuropsychology service.

What happens next?
Most patients recover from delirium as the underlying cause resolves. For some, the effects can last for longer. Many people do not remember their time on critical care

but some people can feel distressed when thinking about this, particularly if they experienced altered perceptions as a result of delirium.
Should you have any concerns about a loved one you can contact the neurocritical care team and an appointment in our post-neurocritical care clinic can be organised. Alternatively, you can organise an appointment to see your GP who can refer you for support.

Where can I get more information?
ICU Steps (www.icusteps.org)
Royal College of Psychiatry (www.rcpsych.ac.uk)
British Geriatrics Society (www.bgs.org.uk)
European Delirium Association (www.eurodeliriumassociation.com)
CIBS centre (icudelirium.org)
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Safety
Kindness
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Improving

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Delirium Family Information Leaflet